INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Tucatinib + capecitabine + trastuzumab

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00936
Version 1

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by intravenous (IV) infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

Treatment stages

01 **Three-medicine treatment**
Repeated 21-day cycles

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

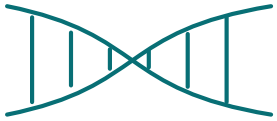
Your medicines and their different roles

The three medicines work together to control previously treated HER2-positive advanced breast cancer. Treatment is reviewed regularly for benefit, symptoms and side effects.

HER2 TYROSINE KINASE INHIBITOR

Tucatinib

Tukysa · oral targeted treatment



Blocks HER2 signals inside cells.

Tucatinib reduces signalling from the internal part of HER2. It works alongside trastuzumab and capecitabine, which act in different ways. Its tablet schedule continues through the capecitabine break.

Reach inside the cell → Block HER2 signalling → Reduce growth signals

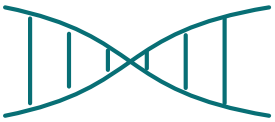
What to watch for

Report diarrhoea early, especially with poor intake. Liver tests and a full medicines review are important.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

The body converts capecitabine into fluorouracil, which disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA processes needed for cell growth. Tablets are active chemotherapy; taking them at home still requires a clear monitoring and interruption plan.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Attend planned echoes. Report breathlessness, ankle swelling, rapid weight gain or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

Indication

Advanced / metastatic · previously treated HER2-positive breast cancer

Where this fits

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by the agreed intravenous (IV) or locally approved subcutaneous (SC) route. Continue while beneficial and tolerated, with clinician-approved changes.

HER2CLIMB · longer follow-up

This explains the study setting and its meaning. Ask the team how the evidence applies to you.

Adding tucatinib helped delay cancer growth and improved survival compared with the other study group. The study does not predict an individual outcome or compare this combination with every newer treatment.

CHAPTER 02

Your schedule, tests and appointments

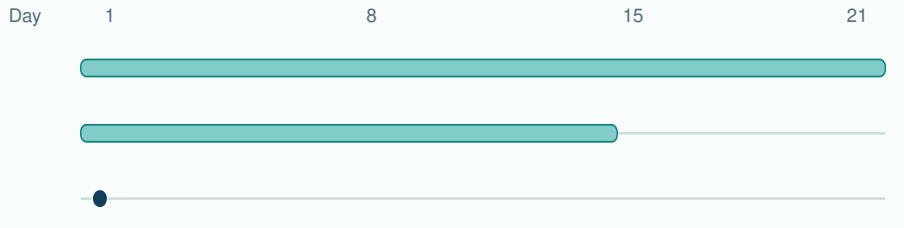
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Each phase in your treatment plan

Tucatinib + capecitabine + trastuzumab

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

REPEATED 21-DAY CYCLES

Three-medicine treatment

Repeated 21-day cycles

The capecitabine break does not mean a tucatinib break. Your team may hold or adjust one medicine without applying the same change to all three. Trastuzumab route is agreed separately; the fixed subcutaneous (SC) dose must not be treated as an intravenous (IV) loading or maintenance dose.

The order depends on the indication and your recovery. The whole-treatment map shows how local treatment and ongoing medicines relate to each other.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Elapsed-time tests remain due if a cycle is delayed. The treatment team books, reviews and acts on results. Calendar timing alone does not authorise a dose.

Check	Before starting	Every 3-week cycle	Around week 12	Ongoing treatment	Treatment held / changed
Blood count / kidney function	full blood count (FBC), renal profile and creatinine clearance	FBC and renal profile before each cycle; sooner for diarrhoea, dehydration or infection	Continue each-cycle tests	Interpret creatinine changes with hydration and true renal function; tucatinib can affect creatinine transport	Reassess renal function and capecitabine safety before restarting

Check	Before starting	Every 3-week cycle	Around week 12	Ongoing treatment	Treatment held / changed
Liver tests	alanine aminotransferase (ALT), aspartate aminotransferase (AST) and bilirubin	Every 3 weeks, or sooner if symptoms / abnormalities	Continue every 3 weeks	Report jaundice or dark urine promptly	Repeat abnormal tests and review tucatinib/ capecitabine separately
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / capecitabine safety	Confirm pretreatment DPD result before first capecitabine; renal impairment can affect uracil phenotyping	Review early severe diarrhoea, mouth ulcers, infection or neurological symptoms immediately	No routine repeat DPD test simply because a cycle passes	Complete deficiency excludes capecitabine; partial deficiency requires an individual plan	Hold affected tablets and seek urgent advice; do not restart independently
Heart function	Cardiac history, electrocardiogram (ECG) and baseline echo/ multigated acquisition (heart pumping) scan (MUGA) under trastuzumab product information	Review breathlessness, swelling and chest pain at each visit	Repeat left ventricular ejection fraction (heart pumping measurement) (LVEF) approximately 12-weekly under the NCCP/ individual cardiac plan	4–8-weekly if impairment; keep elapsed-time due dates when route changes	A qualifying LVEF decline requires hold and repeat within 3 weeks; post-treatment follow-up is planned separately
Diarrhoea / skin / mouth	Provide separate tablet-hold instructions and hydration plan	Review bowels, intake, hands/ feet and mouth before continuation	Symptoms can occur between visits and during capecitabine breaks	Tucatinib continues on the planned Days 15–21 capecitabine break unless specifically held	The medicines have different interruption and restart rules

Check	Before starting	Every 3-week cycle	Around week 12	Ongoing treatment	Treatment held / changed
Disease / central nervous system (CNS) review	Baseline disease assessment and CNS plan when indicated	Clinical review; imaging not tied to tablet-break days	Response assessment commonly every 2–4 months, individualised	Earlier assessment for new symptoms; CNS schedule decided separately	Markers support clinical/ imaging assessment, not decisions alone

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00936 v1 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · response assessment · EMA Tukysa product information · retrieved 25 October 2026 · EMA Xeloda product information · retrieved 25 October 2026 · EMA Herceptin product information · retrieved 25 October 2026

THE WHOLE TREATMENT JOURNEY

Tucatinib + capecitabine + trastuzumab: treatment and checks

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by IV infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–14	Days 15–21	Next cycle	Disease review
Cancer medicines NCCP 00936	Confirm prior HER2 therapies and DPD result	Tucatinib + capecitabine; trastuzumab Day 1 by agreed IV or SC formulation	Tucatinib continues; capecitabine break	Review before any restart / next infusion	Continue, modify or change treatment according to benefit / toxicity
Surgery METASTATIC	No routine curative surgery sequence from this regimen	Local procedures only if indicated	Coordinate interruptions with multidisciplinary team (MDT)	Review symptoms and functional needs	Site-specific / CNS MDT when needed
Radiotherapy METASTATIC	Assess need for symptom or brain-directed treatment	Any concurrent radiotherapy (RT) needs an explicit plan	Do not assume capecitabine can continue with RT	Coordinate holds / restart with radiation team	Imaging and local treatment depend on disease site

Care / check	Before starting	Days 1–14	Days 15–21	Next cycle	Disease review
Blood tests NCCP 00936	FBC, renal, liver; DPD testing before capecitabine	Extra tests if symptoms / toxicity	Pre-cycle bloods and symptom review	FBC / renal per cycle; liver at least every 3 weeks	Check dehydration, hepatic toxicity and interactions
Heart checks NCCP 00936 · HERCEPTIN · ESC	Baseline cardiac assessment, ECG and echo/MUGA	Report chest pain or breathlessness	Symptoms need assessment between visits	Follow the recorded LVEF plan, approximately every 12 weeks	Risk / abnormalities may shorten interval
Cancer imaging METASTATIC · MARKERS	Baseline disease sites; brain assessment as indicated	No scan required simply because Day 14 is reached	Imaging schedule independent of tablet break	Usually review response about every 2–4 months; individualise	Use consistent modalities; earlier imaging if concern

What each check is for

1

Blood tests and treatment review

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review → FBC / renal per cycle; liver at least every 3 weeks → Check dehydration, hepatic toxicity and interactions

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

2

Heart tests

Baseline echo / LVEF → Report chest pain or breathlessness → Symptoms need assessment between visits → Repeat echo according to HER2 plan, commonly 12-weekly → Risk / abnormalities may shorten interval

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

3

Imaging and markers

Baseline disease sites; brain assessment as indicated → No scan required simply because Day 14 is reached → Imaging schedule independent of tablet break → Usually review response about every 2–4 months; individualise → Use consistent modalities; earlier imaging if concern

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

4

Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

5

Trastuzumab formulation confirmed

Before first dose and any route change

IV and SC doses and missed-dose instructions differ.

Ask / act: Confirm the route and visit/observation plan with the team.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before dispensing

Confirm blood tests, interactions, the exact tablet strengths and the plan for missed doses.

STEP 2

Taking tablets

STEP 3

Review before restarting

The team confirms blood results, side effects and the next prescription. Never double a missed dose or restart held treatment without advice.

STEP 4

Trastuzumab visit

Trastuzumab every three weeks with staff observation. IV treatment may use a cannula, peripherally inserted central catheter (PICC) or port; the locally approved SC alternative uses an injection under the skin; no routine take-home infusion pump.

STEP 5

Confirm the trastuzumab route

Your team confirms IV infusion or the locally approved under-the-skin injection. The visit length, observation and missed-dose instructions differ; ask before switching. The injection has no loading dose.

Missed or vomited dose

Do not take an extra dose. Follow the medicine-specific instructions and ask the team if uncertain.

Safe handling

Swallow tablets whole, keep them in their original packaging away from children, and return unused tablets to pharmacy.

Practical arrangements

Confirm who supplies the tablets, who arranges tests, who reviews results and your urgent contact route.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Before starting and between visits

Plan the tests, access and support before the first dose.

Blood tests

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review → FBC / renal per cycle; liver at least every 3 weeks → Check dehydration, hepatic toxicity and interactions

Heart checks

Baseline echo / LVEF → Report chest pain or breathlessness → Symptoms need assessment between visits → Repeat echo according to HER2 plan, commonly 12-weekly → Risk / abnormalities may shorten interval

Imaging, local treatment and follow-up

The whole pathway has several connected parts.

Scans and imaging

Baseline disease sites; brain assessment as indicated → No scan required simply because Day 14 is reached → Imaging schedule independent of tablet break → Usually review response about every 2–4 months; individualise → Use consistent modalities; earlier imaging if concern

Radiotherapy relationship

Assess need for symptom or brain-directed treatment → Any concurrent RT needs an explicit plan → Do not assume capecitabine can continue with RT → Coordinate holds / restart with radiation team → Imaging and local treatment depend on disease site

Bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline dual-energy X-ray absorptiometry (bone density) (DEXA) as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Tucatinib	 **Days 1–21** Twice daily, continuous
Capecitabine ON	 **Days 1–14** Twice daily
Capecitabine OFF	 **Days 15–21** 7-day break
Trastuzumab	 **Day 1** IV or agreed SC formulation
Diarrhoea / hand-foot	 **Days 1–21** Report early, including during break
Pre-cycle review	 **Days 19–21** Timing arranged by unit

Treatment bands: Show the planned medicine schedule, subject to review and dose changes.

Checks and restart: Keep the arranged blood / heart appointments. A calendar day alone does not authorise the next dose.

Symptoms: Can occur at any time; broad bands are not measured risk curves.

Why these bands stay broad: Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00936 v1 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Between-treatment notes

Day 1–21: Your reference cycle

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by IV infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes. Check the agreed prescription and contact the team for new symptoms. Restart only after the team confirms the plan.

Support around the medicines

1 **Support for this phase**

Obtain a clear diarrhoea action plan and separate instructions for each tablet. Anti-sickness medicines and interactions require individual review.

2 **Bloods and cardiac checks**

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review. Symptoms need assessment between visits.

3 **Visits and practical support**

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

Questions worth asking

When are my next tests?

Your team should confirm the next test, who books it, and who reviews the result before treatment. The map shows expected intervals; it does not track completed tests.

Does every scan follow a fixed schedule?

Use consistent modalities; earlier imaging if concern. New symptoms can require earlier assessment.

Try explaining the plan back: Which medicines are you taking now? What changes in the next phase? Which checks are due and who reviews them before treatment?

Tucatinib

- Diarrhoea and dehydration
- Liver test changes or liver injury
- Nausea and fatigue
- Clinically important medicine interactions

Capecitabine

- Hand-foot syndrome
- Mouth ulcers / reduced intake
- Low blood counts and infection
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-related toxicity
- Renal impairment and medicine interactions

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Tucatinib + capecitabine + trastuzumab

Active medicines: Tucatinib + Capecitabine + Trastuzumab

Emetogenic risk: Tucatinib / capecitabine oral minimal to low; trastuzumab minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Agree a personalised antiemetic and diarrhoea prescription; review CYP and other interactions. No repeated intravenous (IV) chemotherapy premedication is assumed for these tablets.
- Tucatinib continues every day; capecitabine is Days 1–14 then off Days 15–21. Confirm separate hold / restart instructions.
- Premedicines are not usually required. If prior hypersensitivity, consider paracetamol / antihistamine under the clinician plan.
- Explain when to hold capecitabine and call for diarrhoea, mouth soreness or painful hand-foot symptoms. Assess dehydration promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Tucatinib + capecitabine + trastuzumab

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by intravenous (IV) infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Diarrhoea / dehydration

Tucatinib + capecitabine

Report diarrhoea early. Follow the separate tablet-hold instructions; reduced urine or dizziness needs prompt advice.

2 Hands / feet

Capecitabine

Painful redness or peeling affecting activity: stop capecitabine and contact oncology.

3 Liver injury

Tucatinib

Keep three-weekly liver blood tests. Report yellow skin / eyes or dark urine.

4 Heart / chest symptoms

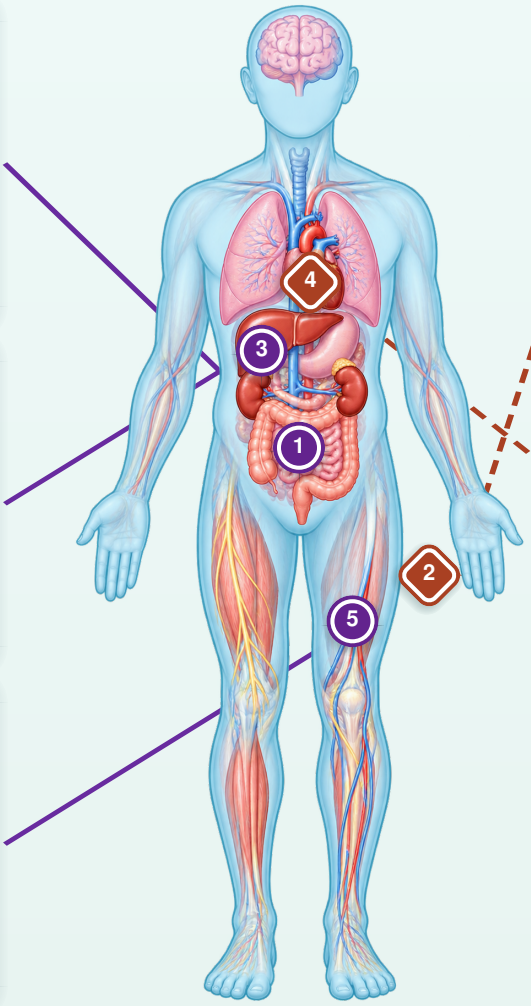
Trastuzumab / capecitabine

Keep echoes. New chest pain: stop capecitabine and seek urgent assessment.

5 Blood counts / infection

Capecitabine

Fever, shivers or sudden illness: call promptly. Blood tests guide the next cycle.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00936 v1 · official regimen ↗**
   https://healthservice.hse.ie/documents/8391/936_Tucatinib_Regimen.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **EMA Tukysa product information · retrieved 25 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/tukysa-epar-product-information_en.pdf
6. **EMA Xeloda product information · retrieved 25 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/xeloda-epar-product-information_en.pdf
7. **EMA Herceptin product information · retrieved 25 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NICE NG101 · bone health and follow-up**
    https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
12. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
13. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
14. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
15. **HER2CLIMB · original analysis (2020)**
    https://pubmed.ncbi.nlm.nih.gov/31825569/
16. **HER2CLIMB final overall survival analysis (2022)**
    https://pubmed.ncbi.nlm.nih.gov/34954044/